
CONSULT FOR AFRICA / HEALTHCARE TRANSFORMATION

THE AESTHETICS PILOT / MEDLYFE WELLNESS x KP x C4A

The Aesthetics Pilot

A light July to November collaboration, before we decide on anything bigger.

PREPARED FOR

Dr Itunu Akinware (Medlyfe Wellness / Medbury) and Dr Chinwe Kpaduwa

BY

Consult for Africa

PERIOD

Pilot: July to November 2026

STATUS

Heads of terms / Draft for discussion

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01 / THE PILOT IN ONE LINE

Test it together, keep it light.

From July to November, Dr Chinwe Kpaduwa runs a premium non-surgical aesthetics service inside Medbury's Medlyfe Wellness brand, at the iFitness Admiralty clinic, with Consult for Africa structuring and running the operation. No standalone company, no capital commitment, no long tie-in. The pilot exists to answer one question: does this work, commercially and between us, well enough to build something bigger after November.

The spirit of it: we start where we are, on infrastructure Medbury already has, and we test each other. Everyone keeps their freedom until November; everyone builds real evidence before anyone commits capital.

And the spirit beyond the terms: this is a partnership of support. Dr Kpaduwa brings rare talent through a season of real growth, a new practice and a new family arriving at once. Medlyfe brings the experience, the platform and the backing to help her thrive. Held with care, that support is what turns a four-month pilot into a partnership that lasts.



02 / THE PARTIES AND WHAT EACH BRINGS

Three roles, no one overcommitted.

Party	What they bring to the pilot
Medlyfe Wellness (Medbury)	The venue, the Medlyfe brand and Regenerative Aesthetics service line, the existing patient and corporate base, and the local team.
Dr Chinwe Kpaduwa	The clinical standard: protocols, training and credentialing of the team, and hands-on delivery of signature treatments while resident and on visits.
Consult for Africa	Structures the arrangement, stands up the operating model, trains and supports the team, and runs the day-to-day and the marketing.



03 / HOW DR CHINWE IS INTRODUCED

Credible to patients, light in commitment.

Patients need to trust the hand behind the service, and Dr Kpaduwa's credentials are the draw. But the introduction must match the reality of a visiting arrangement, so it is honest and carries no overcommitment on either side.

- **Title:** Consultant Plastic and Aesthetic Surgeon, Aesthetics Lead (Visiting), and a founding member of the Medlyfe Advisory Board.
- **The story:** she designs the protocols and the standard, trains and credentials the clinical team, and personally delivers signature treatments during her time in Lagos, so patients get her standard continuously and her hands on her visits.
- **What it avoids:** any claim of permanent daily presence. The visiting-plus-trained-team framing is both true and reassuring, and it protects everyone if plans change.



04 / SCOPE AND VENUE

Non-surgical aesthetics, at the iFitness clinic.

The pilot is non-surgical only, injectables, skin, and regenerative aesthetics, delivered under the Medlyfe Regenerative Aesthetics service. Surgery is out of scope for the pilot; it belongs to the post-November conversation. The venue is Medbury's existing aesthetic clinic at iFitness, Admiralty, Lekki, a decent, ready space that lets us start without a build.

Debo's longer-term preference is a dedicated address that channels its own footfall; for the pilot, the iFitness clinic is the right, low-fidelity starting point, and it keeps the test cheap.



05 / COMMITMENT AND AVAILABILITY

Honest about the diary, covered in the contract.

Dr Kpaduwa is in Nigeria through November and, thereafter, has committed to returning roughly every three months. The clinical standard is hers; the frequency of her visits will, in practice, follow the patient pool. Because life is unpredictable, the arrangement is written so the service does not depend on her being in the building on any given day.

- Her presence is scheduled in blocks around demand, and her visit cadence is captured in the terms rather than left to goodwill.
- Between visits, the trained local team delivers the core menu to her protocols, with her on tele-support for complex cases.



06 / CONTINUITY

The service must stand on its own. This is the point.

This is the term that matters most to Medbury, and it is right. The pilot is built so that if Dr Kpaduwa cannot continue, for any reason, the service continues without disruption. We are building a capability, not building around a person.

- **The team is trained and credentialed** to deliver the core service independently, so capability lives in the clinic, not only in one diary.
- **Medlyfe owns the brand, the systems, the patient data and the protocols.** They stay with Medbury regardless of what any individual decides.
- **No single point of failure.** If she steps back, Medlyfe continues the service internally with the team already trained; nothing collapses.



07 / ECONOMICS OF THE PILOT

Light, revenue-based, no capital at risk.

No one puts in capital for the pilot. Medlyfe provides the venue, the brand and the team; C4A runs it; Dr Kpaduwa earns from the clinical work she and her protocols generate. The simplest fair shape is a share of the aesthetics revenue, split between the clinical draw, the operating cost, and the platform, with the exact percentages set before launch and reviewed at November.

Deliberately simple: revenue in, shared three ways by role, no capital, no debt, no lock-in. The heavier economics, capital, fees and equity, belong to the post-November SPV, not here.

**08 / THE NOVEMBER REVIEW****A clear gate, and the path beyond it.**

In November the three parties review the evidence, demand, delivery, unit economics, and how well we have worked together, and choose one of two paths.

Path	What happens
If it works	Form a standalone SPV, a Medical Aesthetics and Plastic Surgery Centre, which also provides the aesthetics service to Medlyfe under an outsourcing arrangement (so there is continuity and no conflict of interest). Dr Kpaduwa co-invests capital; the fuller structure, capital, fees, equity and protections, is settled then.
If it does not	Medlyfe continues the aesthetics service internally with the trained team, under its own brand. Nothing is stranded, and the relationship stays open.



09 / IF PLANS CHANGE

A graceful exit for everyone.

If Dr Kpaduwa cannot continue, whether through relocation, family, or simply choosing not to, the pilot ends cleanly: the service continues under Medlyfe with the trained team, the brand and data remain Medlyfe's, and each party walks away without loss or claim on the other. A short confidentiality and non-solicitation understanding protects everyone during and just after the pilot. No one is trapped, and no one is left exposed.

**10 / NEXT STEPS****Small, fast, this week.**

- Confirm these heads of terms and the pilot revenue split.
- Debo views the iFitness Admiralty clinic and signs off the space.
- C4A finalises the operating model and launch plan (the companion document).
- Introduce Dr Kpaduwa under the agreed title, train the team, and soft-launch to the Medlyfe and Medbury base.

Consult for Africa Structuring and operating partner

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Draft heads of terms for discussion between Dr Itunu Akinware, Dr Chinwe Kpaduwa and Consult for Africa. Not a binding agreement; terms to be confirmed in writing.